

Alexander J. Ryu
Mayo Clinic
Rochester, Minnesota, USA

August 30, 2026

Editors
npj Digital Medicine

Dear Editors,

Please consider our revised Perspective, “Governance Requirements for Coordinating Clinical AI Agents,” by Bruce Changlong Xu, Ayush Chopra, and Alexander J. Ryu.

Clinical AI systems are moving from isolated recommendations toward actions that intersect across patients, workflows, institutions, and finite resources. The manuscript argues that safety therefore cannot be inferred from individual-agent performance alone. It introduces Clinical Infrastructure for Reliable Coordination (CIRC), a non-exhaustive taxonomy of agent-local, interaction, and institutional risks linked to independent governance controls and observable evidence.

The revision responds directly to the peer reviews. We replaced the original maturity levels with independent governance levers; removed the toy simulations, oncology vignette, and population-level clinical prediction claim; added detailed crosswalks to existing standards and alternative architectures; specified coordination-layer failures, degraded operation, and operational human oversight; and added concrete definitions, a structured-message example, and regulatory boundaries. The manuscript now states explicitly that CIRC is neither a standard nor a validated architecture and that CliniCARE-Bench illustrates only trace-observable agent-local risk.

We believe the revised manuscript fits the journal’s Perspective format because it offers a balanced, evidence-based, and forward-looking framework intended to stimulate discussion and new experimental approaches. The manuscript contains a 69-word abstract, 2,961 words of main prose, 29 references, six tables, two figures, and one illustrative structured-message listing. A detailed point-by-point response accompanies this submission.

Thank you for considering the revised manuscript.

Sincerely,

Alexander J. Ryu
Corresponding author
Mayo Clinic
Ryu.Alexander@mayo.edu